5% 50% 90% 100%

this large trial (N = 555), the authors report that while both risperidone and haloperidol were equally effective in reducing psychosis, risperidone was favored in a couple ways. There was a lower relapse rate (42% vs 55%) with risperidone versus haloperidol, and it was over twice as long before the relapses occurred (466 vs 205 days). Also, not surprisingly, there were more extrapyramidal signs in the haloperidol group. Although there was less weight gain with haloperidol in early treatment, at end point they were essentially the same.

The advantage in extrapyramidal symptoms is undeniable between the traditional and atypical antipsychotics. The kind of results that are evolving are being reported in articles like this one and present a complex set of issues for clinicians. The 13% difference in relapse favoring the atypicals is substantial, certainly for each and every one of those patients and their families. The same can be said for patients who are seriously burdened by, or stopped medicines because of extrapyramidal symptoms, and definitely for the greater percentage who develop tardive dyskinesia with the traditional antipsychotics. However, the policy argument of the increased costs associated with the use of atypicals is heavily weighted in this policy debate, as it should be. However, what is the human and financial cost of relapse? Noncompliance? Tardive dyskinesia?

J. C. Ballenger, MD

Long-term Maintenance Therapy With Quetiapine Versus Haloperidol Decanoate in Patients With Schizophrenia or Schizoaffective Disorder

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Objective.—To compare the long-term efficacy and tolerability of oral quetiapine with those of intramuscular haloperidol.

Method.—Patients with DSM-IV–diagnosed schizophrenia or schizoaffective disorder requiring long-term antipsychotic treatment were randomly assigned to open-label oral quetiapine or intramuscular haloperidol decanoate for 48 weeks. Clinicians were instructed to target dosing at 500 mg/day of quetiapine or 200 mg of haloperidol decanoate every 4 weeks. The Positive and Negative Syndrome Scale was used to assess efficacy; the Simpson-Angus Scale and the Barnes Akathisia Scale were used to assess safety and tolerability. For statistical analyses, a general linear mixed-model repeated-measures analysis of covariance was used, with change scores for dependent variables computed with the baseline score as covariate. Data were collected from 1998 to 2001.

Results.—Thirty-five patients were enrolled, but 6 did not participate after being informed of their treatment assignment; 4 of the 6 withdrawals were assigned to haloperidol decanoate. Mean doses at week 48 were 493 mg/day of quetiapine (N = 16) and 170 mg/28 days of haloperidol decanoate (N = 9). Survival analysis showed no between-group differences in estimates of the number of patients remaining exacerbation-free over time. Both drugs were efficacious, but quetiapine was significantly better than haloper-

5% 50% 90% 100%

idol decanoate in controlling negative symptoms ($p < .05$). The incidence of extrapyramidal symptoms was low in both groups; patients receiving quetiapine showed significantly greater improvement in rigidity and akathisia ($p < .05$).

Conclusion.—Oral quetiapine was as efficacious as intramuscular haloperidol in preventing symptom exacerbation over 48 weeks in patients with schizophrenia or schizoaffective disorder, with fewer extrapyramidal symptoms, especially rigidity and akathisia. Quetiapine was more efficacious than haloperidol decanoate in treating negative symptoms.

► In this 48-week trial, oral quetiapine actually was as effective as intramuscular haloperidol in preventing relapse over a 1-year trial. Perhaps not surprisingly, it also was more efficacious in reducing negative symptoms and certainly had lower extrapyramidal symptoms.

Results in the previous article (Abstract 6–49) finding a lower relapse rate with an atypical compared with oral traditional antipsychotics were surprising enough. This trial actually shows that an oral atypical actually beat intramuscular haloperidol decanoate in preventing relapse, certainly a surprising result.

J. C. Ballenger, MD

Glucose Metabolism in Patients With Schizophrenia Treated With Atypical Antipsychotic Agents: A Frequently Sampled Intravenous Glucose Tolerance Test and Minimal Model Analysis

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Arch Gen Psychiatry 62:19-28, 2005 6–51

Background.—While the incidence of new-onset diabetes mellitus may be increasing in patients with schizophrenia treated with certain atypical antipsychotic agents, it remains unclear whether atypical agents are directly affecting glucose metabolism or simply increasing known risk factors for diabetes.

Objective.—To study the 2 drugs most clearly implicated (clozapine and olanzapine) and risperidone using a frequently sampled intravenous glucose tolerance test.

Design.—A cross-sectional design in stable, treated patients with schizophrenia evaluated using a frequently sampled intravenous glucose tolerance test and the Bergman minimal model analysis.

Setting.—Subjects were recruited from an urban community mental health clinic and were studied at a general clinical research center.

Patients.—Fifty subjects signed informed consent and 41 underwent the frequently sampled intravenous glucose tolerance test. Thirty-six nonobese subjects with schizophrenia or schizoaffective disorder, matched by body mass index and treated with either clozapine, olanzapine, or risperidone, were included in the analysis.